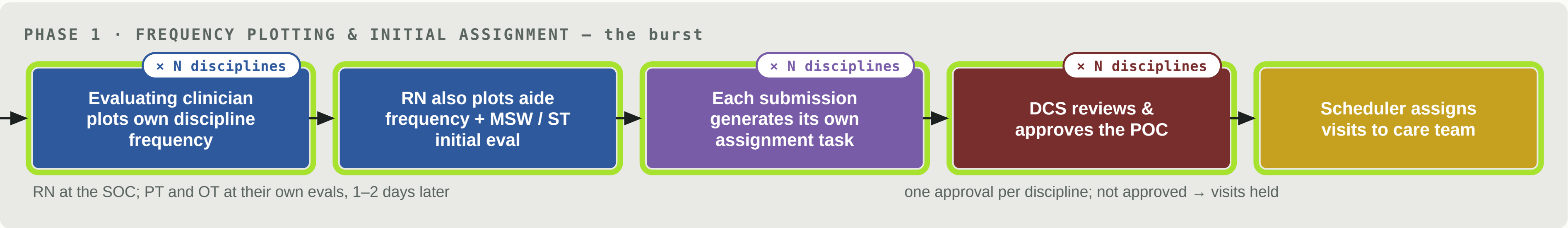
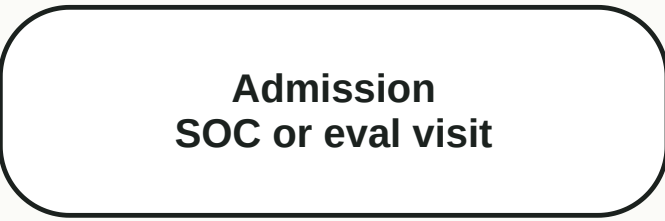


Routine Visit Scheduling

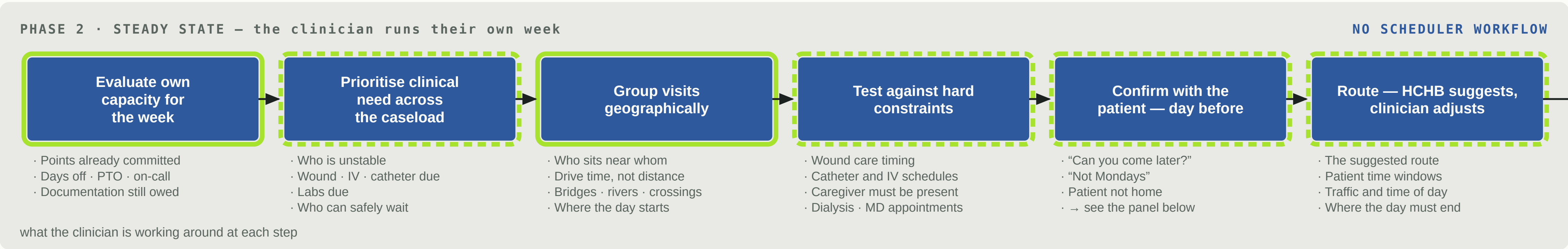
A burst of scheduler work at admission, then the clinician runs their own week

TRIGGER



485 SUBMITTED – changes come by physician order

a physician order may add a discipline evaluation, change the frequency, or reduce the visits every one routes through DCS approval and auth before reaching the scheduler



THE DAY-BEFORE NEGOTIATION – what the clinician has to hold

HARD – accept it and build the day around it

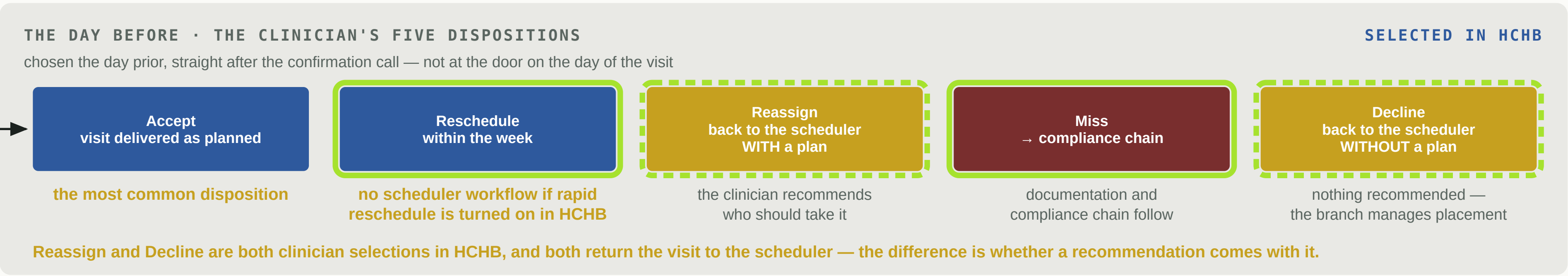
- Dialysis days and times
- MD and specialist appointments
- Caregiver's working hours
- Patient genuinely not home

SOFT – negotiable, and worth holding the line on

- “Can you come after lunch?”
- “Not first thing” · “Not Mondays”
- A preferred time with no reason behind it

The first visit at 8 or 9am is the single largest lever on an individual clinician's capacity.

Newer clinicians let the patient set the time, become over-accommodating, and push the cost onto the rest of the team.



BOUNDARIES

OASIS visits do not move

Medicare week is Sunday–Saturday

Inside the 60-day certification period

Auth still gates assignment

WHERE IT
BREAKS

RN cannot reassign to her own LPN
an HCHB rule, not Medicare

Supervisor cannot see
the supervisee's schedule

Day-before calls are
unpaid evening work

The weekly logic is
undocumented and unassisted